

HOUSE No. 5276

The Commonwealth of Massachusetts

HOUSE OF REPRESENTATIVES, March 25, 2026.

The committee on Mental Health, Substance Use and Recovery, to whom was referred the petition (accompanied by bill, House, No. 2199) of Marjorie C. Decker relative to the involuntary hospitalization, reports recommending that the accompanying bill (House, No. 5276) ought to pass [Representatives Xiarhos of Barnstable, Muradian of Grafton and Howitt of Seekonk dissent].

For the committee,

MINDY DOMB.

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In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act ending unnecessary hospitalizations.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Section 1 of Chapter 123 of the General Laws, as appearing in the 2024
2 Official Edition, is hereby amended by inserting after the definition of “Commissioner” the
3 following definition:-

4 “Community Alternative,” voluntary treatment, services or supports provided in
5 community-based settings as an alternative to hospitalization that include, but are not limited to:
6 (i) crisis services provided through mobile crisis intervention, behavioral health urgent care and
7 community crisis stabilization programs and other voluntary services provided at or by
8 community behavioral health centers as defined in section 16EE of chapter 6A; (ii) MassHealth
9 Behavioral Health (“BH”) Urgent Care; (iii) services offered through the children’s behavioral
10 health initiative or behavioral health services for children and adolescents; (iv) technologically
11 supported behavioral health services and supports, including, but not limited to, the behavioral
12 health helpline and the federally-designated 988 suicide prevention and behavioral health crisis
13 hotline; (v) peer respite and other peer-run programs and services; and (vi) additional services,
14 supports or settings so designated by the department.

SECTION 2. Section 12 of said chapter 123, as so appearing, is hereby amended by striking subsection (a) and inserting in place thereof the following subsection:-

(a) Prior to restraining or authorizing the restraint of a person and applying for the hospitalization of a person under this section, an individual authorized to restrain or authorize the restraint of a person and apply for such hospitalization must first determine that there is no appropriate community alternative for the person. If there is an appropriate community alternative and the person consents to such community alternative, the applicant shall, if applicable, arrange for transport, by ambulance or otherwise, to the community alternative. A physician who is licensed pursuant to section 2 of chapter 112, an advanced practice registered nurse authorized to practice as such under regulations promulgated pursuant to section 80B of said chapter 112, a qualified psychologist licensed pursuant to sections 118 to 129, inclusive, of said chapter 112 or a licensed independent clinical social worker licensed pursuant to sections 130 to 137, inclusive, of said chapter 112 who, after examining a person, has reason to believe that failure to hospitalize such person would create a likelihood of serious harm by reason of mental illness may, upon determining there is no appropriate community alternative or upon determining there is an appropriate community alternative but the person does not consent to such appropriate community alternative, restrain or authorize the restraint of such person and apply for the hospitalization of such person for a 3-day period at a public facility or at a private facility authorized for such purposes by the department. If an examination is not possible because of the emergency nature of the case and because of the refusal of the person to consent to such examination, the physician, qualified psychologist, qualified advanced practice registered nurse or licensed independent clinical social worker on the basis of the facts and circumstances may, after determining that there is no appropriate community alternative or after determining

38 there is an appropriate community alternative but the person does not consent to such appropriate
39 community alternative, determine that hospitalization is necessary and may therefore apply. In an
40 emergency situation, if a physician, qualified psychologist, qualified advanced practice
41 registered nurse or licensed independent clinical social worker is not available, a police officer
42 who, after determining that there is no appropriate community alternative or after determining
43 there is an appropriate community alternative but the person does not consent to such appropriate
44 community alternative, believes that failure to hospitalize a person would create a likelihood of
45 serious harm by reason of mental illness may restrain such person and apply for the
46 hospitalization of such person for a 3-day period at a public facility or a private facility
47 authorized for such purpose by the department. An application for hospitalization shall state: the
48 community alternatives determined to be inappropriate; where applicable, the appropriate
49 community alternatives determined to be appropriate but to which the person did not consent; the
50 reasons for the restraint of such person; and any other relevant information that may assist the
51 admitting physician or qualified advanced practice registered nurse. Whenever practicable, prior
52 to transporting such person, the applicant shall telephone or otherwise communicate with a
53 facility to describe the circumstances and known clinical history and to determine whether the
54 facility is the proper facility to receive such person and to give notice of any restraint to be used
55 and to determine whether such restraint is necessary.

56 SECTION 3. Section 12 of Chapter 123 of the General Laws is hereby amended adding
57 after subsection (e) the following subsections:-

58 (f) The department shall collect information regarding all applications filed pursuant to
59 this section, including the number of applications and other such information as may be relevant,
60 including, but not limited to, information on the age, gender identity, race, ethnicity, insurance

61 status and diagnosis of individuals subject to such applications. The department shall annually,
62 not later than July 31, report to the house and senate committees on ways and means, the joint
63 committee on public health and the joint committee on mental health, substance use and recovery
64 the number of such applications, other information as may be relevant and any actions the
65 department has taken in response to the information it has received, including any licensing
66 actions; provided, however, that the report shall not include any personally identifiable
67 information of individuals subject to such applications.

68 (g) The department, in consultation with the secretary of health and human services, shall
69 maintain a public-facing website that informs the public of community alternatives and the
70 capacity of such community alternatives. The department shall update the website daily. The
71 website shall provide the following information about each community alternative that has
72 availability and capacity: (i) the name, location, telephone number and website of the community
73 alternative; (ii) information about the types of payment, if any, that the community alternative
74 accepts; and (iii) a description of the types of programs and services provided by the community
75 alternative.